

Name : Gaurav Agarwal(31Y/M)

Date : 30 Aug, 2026

Test Asked : Comprehensive Full Body Checkup With Vitamin D And B12 - 3

Report Status : Complete Report



6^{STEP} quality control to ensure 100% report accuracy



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A wing, 'Dattani Plaza', Near East West Industrial Estate, Safed Pool, Saki Naka,
Andheri (East), Mumbai - 400072

Patient Name : GAURAV AGARWAL(31Y/M)

Referred By : SELF

Address : FLAT NO 402 MATHURA VILLA TAPADIA COMPLEX STATION ROAD
JUGSALAI JAMSHEDPUR - 831006 NEAR GHORA CHOWK LANDMARK:
UNION BANK OF INDIA / HDFC BANK STATION ROAD BRANCH

Tests Done : Comprehensive Full Body Checkup With
Vitamin D and B12 - 3

Report Availability Summary

Note: Please refer to the table below for status of your tests.

✓ 11 Ready

✓ 0 Ready with Cancellation

🔄 0 Processing

✗ 0 Cancelled in Lab

TEST DETAILS

REPORT STATUS

Comprehensive Full Body Checkup With Vitamin D and B12 - 3

Ready ✓

FASTING BLOOD SUGAR(GLUCOSE)

Ready ✓

25-OH VITAMIN D (TOTAL)

Ready ✓

IRON

Ready ✓

VITAMIN B-12

Ready ✓

COMPLETE URINE ANALYSIS

Ready ✓

HBA PROFILE

Ready ✓

HEMOGRAM - 6 PART (DIFF)

Ready ✓

LIVER FUNCTION TESTS

Ready ✓

KIDPRO

Ready ✓

LIPID PROFILE

Ready ✓

T3-T4-USTSH

Ready ✓

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Tests Outside Reference Range

Note: Please refer to the table below for tests outside reference range.

Test Name	Observed Value	Units	Bio. Ref. Interval.
COMPLETE HEMOGRAM			
LYMPHOCYTE	40.1	%	20-40
PLATELET DISTRIBUTION WIDTH(PDW)	16	fL	9.6-15.2
LIPID			
HDL / LDL RATIO	0.32	Ratio	> 0.40
HDL CHOLESTEROL - DIRECT	37	mg/dL	40-60
LDL CHOLESTEROL - DIRECT	117.54	mg/dL	< 100
RENAL			
URIC ACID	8.4	mg/dL	4.2 - 7.3
VITAMIN			
25-OH VITAMIN D (TOTAL)	13.94	ng/mL	30-100

Disclaimer: The above listed is the summary of the parameters with values outside the BRI. For detailed report values, parameter correlation and clinical interpretation, kindly refer to the same in subsequent pages.



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Sample Collected on (SCT) : 30 Aug 2026 08:09
Sample Received on (SRT) : 30 Aug 2026 12:36
Report Released on (RRT) : 30 Aug 2026 13:36
Sample Type | Barcode : URINE | FW516528

TEST NAME	METHODOLOGY	VALUE	UNITS	Bio. Ref. Interval.
Complete Urinogram				
<u>Physical Examination</u>				
VOLUME	Visual Determination	3	mL	-
COLOUR	Visual Determination	Straw	-	Pale Yellow
APPEARANCE	Visual Determination	CLEAR	-	Clear
SPECIFIC GRAVITY	pKa change	1.015	-	1.003-1.030
PH	pH indicator	5.5	-	5-8
<u>Chemical Examination</u>				
URINARY PROTEIN	PEI	ABSENT	mg/dL	Absent
URINARY GLUCOSE	GOD-POD	ABSENT	mg/dL	Absent
URINE KETONE	Nitroprusside	ABSENT	mg/dL	Absent
URINARY BILIRUBIN	Diazo coupling	ABSENT	mg/dL	Absent
UROBILINOGEN	Diazo coupling	Normal	mg/dL	<=0.2
BILE SALT	Hays sulphur	ABSENT	-	Absent
BILE PIGMENT	Ehrlich reaction	ABSENT	-	Absent
URINE BLOOD	Peroxidase reaction	ABSENT	-	Absent
NITRITE	Diazo coupling	ABSENT	-	Absent
LEUCOCYTE ESTERASE	Esterase reaction	ABSENT	-	Absent
<u>Microscopic Examination</u>				
MUCUS	Microscopy	ABSENT	-	Absent
RED BLOOD CELLS	Microscopy	ABSENT	cells/HPF	0-2
URINARY LEUCOCYTES (PUS CELLS)	Microscopy	ABSENT	cells/HPF	0-5
EPITHELIAL CELLS	Microscopy	ABSENT	cells/HPF	0-5
CASTS	Microscopy	ABSENT	-	Absent
CRYSTALS	Microscopy	ABSENT	-	Absent
BACTERIA	Microscopy	ABSENT	-	Absent
YEAST	Microscopy	ABSENT	-	Absent
PARASITE	Microscopy	ABSENT	-	Absent

(Reference : *PEI - Protein error of indicator, *GOD-POD - Glucose oxidase-peroxidase)

Tests Done : COMPLETE URINE ANALYSIS





Report Remarks : Clinically Tested by: Thyrocare Technologies Ltd

Dr Jayshree Maity
MD(Path)

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Sample Collected on (SCT) : 30 Aug 2026 08:09
Sample Received on (SRT) : 30 Aug 2026 12:57
Report Released on (RRT) : 30 Aug 2026 14:43
Sample Type | Barcode : EDTA Whole Blood | GF215965

TEST NAME	METHODOLOGY	VALUE	UNITS	Bio. Ref. Interval.
HEMOGLOBIN	Colorimetry	14.8	g/dL	13.0-17.0
Hematocrit (PCV)	CPH Detection	43	%	40.0-50.0
Total RBC	HF & EI	4.72	X 10 ⁶ /μL	4.5-5.5
Mean Corpuscular Volume (MCV)	Calculated	91.2	fL	83.0-101.0
Mean Corpuscular Hemoglobin (MCH)	Calculated	31.2	pq	27.0-32.0
Mean Corp.Hemo. Conc (MCHC)	Calculated	34.4	g/dL	31.5-34.5
Red Cell Distribution Width - SD (RDW-SD)	Calculated	45.2	fL	39-46
Red Cell Distribution Width (RDW - CV)	Calculated	12.9	%	11.6-14
RED CELL DISTRIBUTION WIDTH INDEX (RDWI)	Calculated	249.3	-	*Refer Note below
MENTZER INDEX	Calculated	19.3	-	*Refer Note below
TOTAL LEUCOCYTE COUNT (WBC)	HF & FC	5.9	X 10 ³ / μL	4.0 - 10.0
DIFFERENTIAL LEUCOCYTE COUNT				
Neutrophils Percentage	Flow Cytometry	50.3	%	40-80
Lymphocytes Percentage	Flow Cytometry	40.1	%	20-40
Monocytes Percentage	Flow Cytometry	6.5	%	2-10
Eosinophils Percentage	Flow Cytometry	2.2	%	1-6
Basophils Percentage	Flow Cytometry	0.5	%	0-2
Immature Granulocyte Percentage (IG%)	Flow Cytometry	0.4	%	0-0.5
Nucleated Red Blood Cells %	Flow Cytometry	0.01	%	0.0-5.0
ABSOLUTE LEUCOCYTE COUNT				
Neutrophils - Absolute Count	Calculated	2.97	X 10 ³ / μL	2.0-7.0
Lymphocytes - Absolute Count	Calculated	2.37	X 10 ³ / μL	1.0-3.0
Monocytes - Absolute Count	Calculated	0.38	X 10 ³ / μL	0.2 - 1.0
Basophils - Absolute Count	Calculated	0.03	X 10 ³ / μL	0.02 - 0.1
Eosinophils - Absolute Count	Calculated	0.13	X 10 ³ / μL	0.02 - 0.5
Immature Granulocytes (IG)	Calculated	0.02	X 10 ³ / μL	0-0.3
Nucleated Red Blood Cells	Calculated	0.01	X 10 ³ / μL	0.0-0.5
PLATELET COUNT	HF & EI	264	X 10 ³ / μL	150-410
Mean Platelet Volume (MPV)	Calculated	9.8	fL	6.5-12
Platelet Distribution Width (PDW)	Calculated	16	fL	9.6-15.2
Platelet to Large Cell Ratio (PLCR)	Calculated	23.5	%	19.7-42.4
Plateletcrit (PCT)	Calculated	0.26	%	0.19-0.39

Remarks : Alert!!! Predominantly normocytic normochromic with ovalocytes. Platelets:Appear adequate in smear.

*Note - Mentzer index (MI), RDW-CV and RDWI are hematological indices to differentiate between Iron Deficiency Anemia (IDA) and Beta Thalassemia Trait (BTT). MI >13, RDWI >220 and RDW-CV >14 more likely to be IDA. MI <13, RDWI <220, and RDW-CV <14 more likely to be BTT. Suggested Clinical correlation. BTT to be confirmed with HB electrophoresis if clinically indicated.

Method : Fully automated bidirectional analyser

(Reference : *FC- flowcytometry, *HF- hydrodynamic focussing, *EI- Electric Impedence, *Hb- hemoglobin, *CPH- Cumulative pulse height)

Tests Done : HBA PROFILE,HEMOGRAM - 6 PART (DIFF)

Jayashree Maity



Report Remarks : Clinically Tested by: Thyrocare Technologies Ltd

Dr Jayashree Maity
MD(Path)

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TEST NAME	TECHNOLOGY	VALUE	UNITS
HbA1c	H.P.L.C	5.1	%

Bio. Ref. Interval. :**As per ADA Guidelines**

Below 5.7% : Normal
5.7% - 6.4% : Prediabetic
≥6.5% : Diabetic

Guidance For Known Diabetics

Below 6.5% : Good Control
6.5% - 7% : Fair Control
7.0% - 8% : Unsatisfactory Control
≥8% : Poor Control

Method : Fully Automated H.P.L.C method

AVERAGE BLOOD GLUCOSE (ABG) CALCULATED 100 mg/dL

Bio. Ref. Interval. :

90 - 120 mg/dl : Good Control
121 - 150 mg/dl : Fair Control
151 - 180 mg/dl : Unsatisfactory Control
> 180 mg/dl : Poor Control

Method : Derived from HbA1c values

Please correlate with clinical conditions.

Tests Done : HBA PROFILE,HEMOGRAM - 6 PART (DIFF)

Jayashree Maity



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Sample Collected on (SCT) : 30 Aug 2026 08:09
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Report Released on (RRT) : 30 Aug 2026 16:56
Sample Type | Barcode : SERUM | GC984396

TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval
TOTAL CHOLESTEROL	PHOTOMETRY	180	mg/dL	< 200
HDL CHOLESTEROL - DIRECT	PHOTOMETRY	37	mg/dL	40-60
LDL CHOLESTEROL - DIRECT	PHOTOMETRY	117.54	mg/dL	< 100
TRIGLYCERIDES	PHOTOMETRY	112	mg/dL	< 150
TC/ HDL CHOLESTEROL RATIO	CALCULATED	4.8	Ratio	3 - 5
TRIG / HDL RATIO	CALCULATED	3.01	Ratio	< 3.12
LDL / HDL RATIO	CALCULATED	3.2	Ratio	1.5-3.5
HDL / LDL RATIO	CALCULATED	0.32	Ratio	> 0.40
NON-HDL CHOLESTEROL	CALCULATED	142.52	mg/dL	< 160
VLDL CHOLESTEROL	CALCULATED	22.41	mg/dL	5 - 40

Please correlate with clinical conditions.

Method :

CHOL - Cholesterol Oxidase, Esterase, Peroxidase
HCHO - Direct Enzymatic Colorimetric
LDL - Direct Measure
TRIG - Enzymatic, End Point
TC/H - Derived from serum Cholesterol and Hdl values
TRI/H - Derived from TRIG and HDL Values
LDL/ - Derived from serum HDL and LDL Values
HD/LD - Derived from HDL and LDL values.
NHDH - Derived from serum Cholesterol and HDL values
VLDL - Derived from serum Triglyceride values

***REFERENCE RANGES AS PER NCEP ATP III GUIDELINES:**

TOTAL CHOLESTEROL	(mg/dl)	HDL	(mg/dl)	LDL	(mg/dl)	TRIGLYCERIDES	(mg/dl)
DESIRABLE	<200	LOW	<40	OPTIMAL	<100	NORMAL	<150
BORDERLINE HIGH	200-239	HIGH	>60	NEAR OPTIMAL	100-129	BORDERLINE HIGH	150-199
HIGH	>240			BORDERLINE HIGH	130-159	HIGH	200-499
				HIGH	160-189	VERY HIGH	>500
				VERY HIGH	>190		

Alert !!! 10-12 hours fasting is mandatory for lipid parameters. If not, values might fluctuate.

Tests Done : Comprehensive Full Body Checkup With Vitamin D and
B12 - 3

Report Remarks : Clinically Tested by: Thyrocare Technologies Ltd

Jayashree Maity

Dr Jayashree Maity
MD(Path)



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Sample Type | Barcode : SERUM | GC984396

TEST NAME	TECHNOLOGY	VALUE	UNITS
25-OH VITAMIN D (TOTAL) Bio. Ref. Interval. :-	E.C.L.I.A	13.94	ng/mL

Deficiency : ≤ 20 ng/ml || Insufficiency : 21-29 ng/ml
Sufficiency : ≥ 30 ng/ml || Toxicity : > 100 ng/ml

Clinical Significance:

Vitamin D is a fat soluble vitamin that has been known to help the body absorb and retain calcium and phosphorous; both are critical for building bone health.

Decrease in vitamin D total levels indicate inadequate exposure of sunlight, dietary deficiency, nephrotic syndrome.

Increase in vitamin D total levels indicate Vitamin D intoxication.

Specifications: Precision: Intra assay (%CV):9.20%, Inter assay (%CV):8.50%

Kit Validation Reference : Holick M. Vitamin D the underappreciated D-Lightful hormone that is important for Skeletal and cellular health Curr Opin Endocrinol Diabetes 2002;9(1)87-98.

Please correlate with clinical conditions.

Method:- Fully Automated Electrochemiluminescence Competitive Immunoassay

Tests Done : Comprehensive Full Body Checkup With Vitamin D and B12 - 3

Report Remarks : Clinically Tested by: Thyrocare Technologies Ltd

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TEST NAME	TECHNOLOGY	VALUE	UNITS
VITAMIN B-12 Bio. Ref. Interval. :-	E.C.L.I.A	484	pg/mL

Normal: 197-771 pg/ml

Clinical significance :

Vitamin B12 or cyanocobalamin, is a complex corrinoid compound found exclusively from animal dietary sources, such as meat, eggs and milk. It is critical in normal DNA synthesis, which in turn affects erythrocyte maturation and in the formation of myelin sheath. Vitamin-B12 is used to find out neurological abnormalities and impaired DNA synthesis associated with macrocytic anemias. For diagnostic purpose, results should always be assessed in conjunction with the patients medical history, clinical examination and other findings.

Specifications: Intra assay (%CV):2.6%, Inter assay (%CV):2.3 %

Kit Validation Reference : Thomas L.Clinical laborator Diagnostics : Use and Assessment of Clinical laboratory Results 1st Edition,TH Books-Verl-Ges,1998:424-431

Please correlate with clinical conditions.

Method:- Fully Automated Electrochemiluminescence Compititive Immunoassay

Tests Done : Comprehensive Full Body Checkup With Vitamin D and B12 - 3

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TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval.
TOTAL TRIIODOTHYRONINE (T3)	E.C.L.I.A	92	ng/dL	80-200
TOTAL THYROXINE (T4)	E.C.L.I.A	5.71	µg/dL	4.8-12.7
TSH - ULTRASENSITIVE	E.C.L.I.A	3.03	µIU/mL	0.54-5.30

Comments : IF NOT ON DRUGS SUGGESTED FT3 & FT4 ESTIMATION

The Biological Reference Ranges is specific to the age group. Kindly correlate clinically.

Method :

T3,T4 - Fully Automated Electrochemiluminescence Competitive Immunoassay

USTSH - Fully Automated Electrochemiluminescence Sandwich Immunoassay

Disclaimer : Results should always be interpreted using the reference range provided by the laboratory that performed the test. Different laboratories do tests using different technologies, methods and using different reagents which may cause difference. In reference ranges and hence it is recommended to interpret result with assay specific reference ranges provided in the reports. To diagnose and monitor therapy doses, it is recommended to get tested every time at the same Laboratory.

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TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval
BLOOD UREA NITROGEN (BUN)	PHOTOMETRY	16.06	mg/dL	7.94 - 20.07
CREATININE - SERUM	PHOTOMETRY	1.03	mg/dL	0.72-1.18
BUN / Sr.CREATININE RATIO	CALCULATED	15.59	Ratio	9:1-23:1
UREA (CALCULATED)	CALCULATED	34.37	mg/dL	Adult : 17-43
UREA / SR.CREATININE RATIO	CALCULATED	33.37	Ratio	< 52
CALCIUM	PHOTOMETRY	9.53	mg/dL	8.8-10.6
URIC ACID	PHOTOMETRY	8.4	mg/dL	4.2 - 7.3

Please correlate with clinical conditions.

Method :

BUN - Kinetic UV Assay.

SCRE - Creatinine Enzymatic Method

B/CR - Derived from serum Bun and Creatinine values

UREAC - Derived from BUN Value.

UR/CR - Derived from UREA and Sr.Creatinine values.

CALC - Arsenazo III Method, End Point.

URIC - Uricase / Peroxidase Method

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TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval
ALKALINE PHOSPHATASE	PHOTOMETRY	94.25	U/L	45-129
BILIRUBIN - TOTAL	PHOTOMETRY	0.73	mg/dL	0.3-1.2
BILIRUBIN -DIRECT	PHOTOMETRY	0.17	mg/dL	0 - 0.20
BILIRUBIN (INDIRECT)	CALCULATED	0.56	mg/dL	0-0.9
GAMMA GLUTAMYL TRANSFERASE (GGT)	PHOTOMETRY	23.49	U/L	< 55
ASPARTATE AMINOTRANSFERASE (SGOT)	PHOTOMETRY	27.87	U/L	< 35
ALANINE TRANSAMINASE (SGPT)	PHOTOMETRY	38.5	U/L	< 45
SGOT / SGPT RATIO	CALCULATED	0.72	Ratio	< 2
PROTEIN - TOTAL	PHOTOMETRY	7.8	gm/dL	5.7-8.2
ALBUMIN - SERUM	PHOTOMETRY	4.44	gm/dL	3.2-4.8
SERUM GLOBULIN	CALCULATED	3.36	gm/dL	2.5-3.4
SERUM ALB/GLOBULIN RATIO	CALCULATED	1.32	Ratio	0.9 - 2

Please correlate with clinical conditions.

Method :

ALKP - Modified IFCC method

BILT - Diazonium salt DPD method

BILD - Diazonium salt DPD method

BILI - Derived from serum Total and Direct Bilirubin values

GGT - Modified IFCC method

SGOT - IFCC* Without Pyridoxal Phosphate Activation

SGPT - IFCC* Without Pyridoxal Phosphate Activation

OT/PT - Derived from SGOT and SGPT values.

PROT - Biuret Method

SALB - Albumin Bcg¹method (Colorimetric Assay Endpoint)

SEGB - DERIVED FROM SERUM ALBUMIN AND PROTEIN VALUES

A/GR - Derived from serum Albumin and Protein values

Tests Done : Comprehensive Full Body Checkup With Vitamin D and
B12 - 3

Report Remarks : Clinically Tested by: Thyrocare Technologies Ltd

Jayashree Maity

Dr Jayshree Maity
MD(Path)



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Andheri (East), Mumbai - 400072

Patient Name : GAURAV AGARWAL(31Y/M)

Referred By : SELF

Address : FLAT NO 402 MATHURA VILLA TAPADIA COMPLEX STATION
ROAD JUGSALAI JAMSHEDPUR - 831006 NEAR GHORA
CHOWK LANDMARK: UNION BANK OF INDIA / HDFC BANK

Sample Collected on (SCT) : 30 Aug 2026 08:09

Sample Received on (SRT) : 30 Aug 2026 13:16

Report Released on (RRT) : 30 Aug 2026 16:56

Sample Type | Barcode : SERUM | GC984396

TEST NAME	TECHNOLOGY	VALUE	UNITS
IRON	PHOTOMETRY	90	µg/dL
Bio. Ref. Interval. : Male : 65 - 175 Female : 50 - 170			
Method : Ferrozine method without deproteinization			
Please correlate with clinical conditions.			

Tests Done : Comprehensive Full Body Checkup With Vitamin D and
B12 - 3

Report Remarks : Clinically Tested by: Thyrocare Technologies Ltd

Dr Jayshree Maity
MD(Path)



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Sample Received on (SRT) : 30 Aug 2026 13:16
Report Released on (RRT) : 30 Aug 2026 16:56
Sample Type | Barcode : SERUM | GC984396

TEST NAME	TECHNOLOGY	VALUE	UNITS
EST. GLOMERULAR FILTRATION RATE (eGFR) Bio. Ref. Interval. :-	CALCULATED	100	mL/min/1.73 m2

> = 90 : Normal
60 - 89 : Mild Decrease
45 - 59 : Mild to Moderate Decrease
30 - 44 : Moderate to Severe Decrease
15 - 29 : Severe Decrease
<15 : Kidney Failure

Clinical Significance

The normal serum creatinine reference interval does not necessarily reflect a normal GFR for a patient. Because mild and moderate kidney injury is poorly inferred from serum creatinine alone. Thus, it is recommended for clinical laboratories to routinely estimate glomerular filtration rate (eGFR), a "gold standard" measurement for assessment of renal function, and report the value when serum creatinine is measured for patients 18 and older, when appropriate and feasible. It cannot be measured easily in clinical practice, instead, GFR is estimated from equations using serum creatinine, age, race and sex. This provides easy to interpret information for the doctor and patient on the degree of renal impairment since it approximately equates to the percentage of kidney function remaining. Application of CKD-EPI equation together with the other diagnostic tools in renal medicine will further improve the detection and management of patients with CKD.

Reference

Levey AS, Stevens LA, Schmid CH, Zhang YL, Castro AF, 3rd, Feldman HI, et al. A new equation to estimate glomerular filtration rate. Ann Intern Med. 2009;150(9):604-12.

Please correlate with clinical conditions.

Method:- 2021 CKD EPI Creatinine Equation

Tests Done : Comprehensive Full Body Checkup With Vitamin D and
B12 - 3

Report Remarks : Clinically Tested by: Thyrocare Technologies Ltd

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ROAD JUGSALAI JAMSHEDPUR - 831006 NEAR GHORA
CHOWK LANDMARK: UNION BANK OF INDIA / HDFC BANK

Sample Collected on (SCT) : 30 Aug 2026 08:09
Sample Received on (SRT) : 30 Aug 2026 13:19
Report Released on (RRT) : 30 Aug 2026 14:47
Sample Type | Barcode : FLUORIDE PLASMA | GI576668

TEST NAME	TECHNOLOGY	VALUE	UNITS
FASTING BLOOD SUGAR(GLUCOSE)	PHOTOMETRY	75.55	mg/dL

Bio. Ref. Interval. :-

As per ADA Guideline: Fasting Plasma Glucose (FPG)	
Normal	70 to 100 mg/dl
Prediabetes	100 mg/dl to 125 mg/dl
Diabetes	126 mg/dl or higher

Note :

The assay could be affected mildly and may result in anomalous values if serum samples have heterophilic antibodies, hemolyzed, icteric or lipemic. The concentration of Glucose in a given specimen may vary due to differences in assay methods, calibration and reagent specificity. For diagnostic purposes results should always be assessed in conjunction with patients medical history, clinical findings and other findings.

Please correlate with clinical conditions.

Method:- GOD-POD METHOD

Tests Done : BLOOD SUGAR (F)

Jayashree Maity



Report Remarks : Clinically Tested by: Thyrocare Technologies Ltd

Dr Jayashree Maity
MD(Path)

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CONDITIONS OF REPORTING

- ✓ The reported results are for information and interpretation of the referring doctor only.
- ✓ It is presumed that the tests performed on the specimen belong to the patient; named or identified.
- ✓ Results of tests may vary from laboratory to laboratory and also in some parameters from time to time for the same patient.
- ✓ Should the results indicate an unexpected abnormality, the same should be reconfirmed.
- ✓ Only such medical professionals who understand reporting units, reference ranges and limitations of technologies should interpret results.
- ✓ This report is not valid for medico-legal purpose.
- ✓ Docon Technologies Private Limited, Thyrocare Technologies Limited and its employees/representatives do not assume any liability, responsibility for any loss or damage that may be incurred by any person as a result of presuming the meaning or contents of the report.

EXPLANATIONS

- ✓ **Name** - The name is as declared by the client and recorded by the personnel who collected the specimen.
- ✓ **Ref.By** - The name of the doctor who has recommended testing as declared by the client.
- ✓ **Labcode** - This is the accession number in our laboratory and it helps us in archiving and retrieving the data.
- ✓ **Barcode** - This is the specimen identity number and it states that the results are for the specimen bearing the barcode (irrespective of the name).
- ✓ **SCT** - Specimen Collection Time - The time when specimen was collected as declared by the client.
- ✓ **SRT** - Specimen Receiving Time - This time when the specimen reached our laboratory.
- ✓ **RRT** - Report Releasing Time - The time when our pathologist has released the values for Reporting.
- ✓ **Reference Range** - Means the range of values in which 95% of the normal population would fall.

SUGGESTIONS

- ✓ Values out of reference range requires reconfirmation before starting any medical treatment.
- ✓ Retesting is needed if you suspect any quality shortcomings.
- ✓ For suggestions, complaints or feedback, write to us at grievance-office@docon.co.in or call us on 7022000900.

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